Written Use Case: Book Appointment

Use Case Title: Book Appointment
Primary Actor: Patient (or Receptionist)
Level: Kite (summary)
Stakeholders: Patient, Physician, Clinic Management
Precondition: Patient exists in the system (or is newly registered)
Minimal Guarantee: Rollback of any uncompleted transaction; no bad data saved
Success Guarantees: Appointment is confirmed and securely logged in the system schedule
Trigger: Patient initiates a booking request online or via phone

Main Success Scenario

1. Patient requests an appointment with a physician for a desired date and time.
2. System displays available time slots for physicians.
3. Patient selects a preferred slot and doctor.
4. System validates 24-hour cancellation and scheduling rules.
5. System confirms the booking and sends confirmation details to the patient.

Extensions

1a. Requested date/time slot is unavailable.
1a1. Patient selects an alternative time slot.
1a2. Patient opts to join the walk-in queue instead.
4a. Booking transaction is interrupted or fails.
4a1. Transaction rolled back. Patient restarts the booking process.

Written Use Case: Check-In Patient & Verify Health Card

Use Case Title: Check-In Patient & Verify Health Card
Primary Actor: Receptionist / Secretary
Level: Kite (summary)
Stakeholders: Patient, Receptionist, Ministry of Health
Precondition: Patient has arrived at the clinic for a scheduled visit or walk-in
Minimal Guarantee: Rollback of any uncompleted check-in transaction; no incorrect patient status recorded
Success Guarantees: Patient status is updated to “Checked-In” and health card is verified in real-time
Trigger: Patient presents at the front desk upon arrival

Main Success Scenario

1. Patient arrives at the front desk and presents their health card.
2. Receptionist enters the health card number into the system.
3. System connects in real-time with the Ministry of Health system to verify coverage.
4. Ministry confirms valid health card status.
5. Receptionist marks the patient’s status as “Checked-In” and assigns them to the examination queue.

Extensions

3a. Ministry system link is offline or unavailable.
3a1. System issues a connection warning.
3a2. Receptionist proceeds with manual temporary verification and flags record for later audit.
4a. Health card is invalid or expired.
4a1. System flags a validation error.
4a2. Receptionist requests updated billing information or secondary identification from the patient.

Written Use Case: Perform Triage & Record Vitals

Use Case Title: Perform Triage & Record Vitals
Primary Actor: Nurse
Level: Kite (summary)
Stakeholders: Nurse, Patient, Physician
Precondition: Patient is checked in at the front desk and brought to the examination/triage area
Minimal Guarantee: Rollback of any incomplete data entry; existing patient records remain uncorrupted
Success Guarantees: Patient vitals, symptom history, and triage priority level are successfully saved in the Clinical Record
Trigger: Nurse calls the patient into the examination room

Main Success Scenario

1. Nurse calls patient into the examination room.
2. Nurse measures and enters temperature, height, weight, and blood pressure into the system.
3. Nurse interviews patient and records a brief history of problems and symptoms.
4. Nurse saves the clinical entry into the system database.
5. System updates the patient's record and routes the file to the physician's examination queue.

Extensions

2a. Critical vitals or urgent symptoms detected (e.g., chest pain).
2a1. Nurse triggers the triage jump/priority queue process.
2a2. System immediately moves the patient to the top of the physician's active queue for urgent care.
4a. Data entry is interrupted or network fails.
4a1. System prompts nurse to re-enter unsaved vitals and rolls back incomplete entries.

Written Use Case: Conduct Consultation & Prescribe Medication

Use Case Title: Conduct Consultation & Prescribe Medication
Primary Actor: Doctor / Physician
Level: Kite (summary)
Stakeholders: Physician, Patient, Pharmacy, Laboratory, Specialist
Precondition: Patient is in the examination room with recorded vitals and symptoms
Minimal Guarantee: Rollback of uncompleted clinical transactions; no unverified prescription or lab data saved
Success Guarantees: Patient visit history, diagnosis, e-prescriptions, and lab/specialist requisitions are securely logged and transmitted
Trigger: Physician begins the consultation with the patient

Main Success Scenario

1. Physician reviews patient history, vitals, and symptoms.
2. Physician conducts the examination and enters complaints, diagnosis, and treatment plans.
3. Physician generates an electronic prescription, lab requisition, or specialist referral.
4. System transmits the prescription electronically to the pharmacy or routes requisitions to external partners.
5. Physician concludes the visit, and receptionist marks the status as "Checked-Out".

Extensions

3a. External third-party pharmacy or lab lacks a compatible electronic interface.
3a1. System defaults to generating an official printout for the patient to hand-carry.
4a. Abnormal lab or test results are detected later.
4a1. System triggers an automated patient recall alert, initiating follow-up calls or messages.

Written Use Case: Process Payment for Uninsured Services

Use Case Title: Process Payment for Uninsured Services

Primary Actor: Receptionist / Secretary (with Patient)

Level: Kite (summary)

Stakeholders: Clinic Management, Patient, Accounting Firm

Precondition: Patient has received an uninsured service that is not covered by government health insurance

Minimal Guarantee: Roll back interrupted transactions without saving unverified payment logs.

Success Guarantees: Patient visit history, diagnosis, e-prescriptions, and lab/specialist requisitions are securely logged and transmitted

Trigger: Patient requests or completes an uninsured service requiring direct payment

Main Success Scenario

1. Physician reviews patient history, vitals, and symptoms. System calculates the fee for non-covered services.
2. Patient selects a payment method (cash or major credit card).
3. Receptionist enters the payment details into the system payment form.
4. System processes the transaction, updates Financial Accounts, and generates a payment receipt.
5. Receptionist provides the receipt to the patient.

Extensions

3a. Credit card payment is declined or fails.

3a1. System issues a decline notification.

3a2. Patient provides an alternative credit card or pays using cash.

4a. Transaction network connection is interrupted.

4a1. System rolls back the incomplete financial entry.

4a2. Receptionist prompts the user to re-initiate the payment transaction.